DPP — DEPARTMENTAL POLICY AND PROCEDURE

TITLE:	ADMISSION CRITERIA
DEPARTMENT:	INTENSIVE CARE UNIT
APPLIES TO:	ICU AND NURSING STAFF

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CONTENTS

1. PURPOSE	4. POLICY	7. REFERENCE/CROSS-REFERENCES
2. DEFINITION & ABBREVIATION	5. PROCEDURE	8. ATTACHMENT
3. RESPONSIBILITY	6. MEASUREMENT	9. REVISION CLAUSE

1. PURPOSE

1.1. To set procedural guidelines on how to admit patient in intensive care unit.

1.2. To determine patient who need ICU care according to priorities in accordance to ICU bed capacity.

1.3. To avoid unnecessary admission of patient in ICU.

1.4. To provide advance monitoring and intensive treatment for all medical and surgical patients

2. DEFINITION & ABBREVIATION

2.1. **Intensive care** – refers to the specialized treatment given to patients who are acutely unwell and require critical medical care.

2.2. **Intensive Care Unit (ICU)** – provides the critical care and life support for acutely ill and injured patients

2.3. **Designee** – is a person selected or designated to carry out a duty or role.

2.4. **NOD** – Nurse on Duty

2.5. **APACHE (Acute Physiology and Chronic Health Evaluation) II SCORE** – Provides a robust method for classifying the severity of illness in critically ill patients.

3. RESPONSIBILITY

3.1. **Medical Director** – ensures that all departments adhere to this policy.

3.2. **Head of ICU**

3.2.1. Overall in-charge of ICU

3.2.2. Ensures that patient meet eligibility requirement for admission.

3.3. **ICU Physician**

3.3.1. Assess and evaluate patient's condition, discuss patient's condition with the head of ICU before admitting the patient.

3.3.2. Manage patient in the requesting unit/ward according to priority

3.3.3. Inform the ICU Charge Nurse once patient is for admission.

3.3.4. APACHE II Score is assessed and documented by the ICU nurse and reviewed and signed by the attending physician.

3.4. **Attending physician and Medical Specialist On Call/Designee** – informs and discusses patient's condition to ICU physicians including history and present condition, and acquire consent for ICU admission with the ICU physician, for further invasive procedures as needed

3.5. **ICU Nurse** – prepares the bed and necessary equipment and/or materials according to patient's condition and needs, assess and evaluate patient condition trough APACHE II score refer to MCC.

3.6. **Unit/ Ward Nurse**

3.6.1. Must inform the patient's present condition to the ICU Nurse prior to shifting the patient to the ICU

3.6.2. Witnesses obtaining of CRITICAL CARE CONSENT FORM by Attending Physician/ Medical Specialist On Call/ Designee in the ward/unit

4. POLICY

4.1. Admission to ICU will be according to criteria and priorities.

4.2. APACHE II

4.2.1. Purpose: Define patient inclusion criteria and timing (usually within the first 24 hours of ICU admission).

4.2.2. Data Collection :

4.2.2.1. **Mandate recording of 12 routine physiological measurements** : (Temperature, Mean Arterial Pressure, Heart Rate, Respiratory Rate, Oxygenation, Arterial PH, Serum Sodium, Serum Potassium, Serum Creatinine, Hematocrit, WBC Count and Glasgow Coma Scale),

4.2.2.2. **Age point**: Points are added based on the patient's age.

4.2.2.3. **Chronic health points**: Points are added if the patient has a history of severe organ system insufficiency or is immunocompromised.

4.2.3. Risk Stratification : Predicts the statistical risk of patient mortality in Intensive care unit

4.3. **Categories of patient qualified for admission or transfer to ICU**

4.3.1. Multiple trauma including e.g. head injuries

4.3.2. Penetrating wounds in the chest and abdomen that are hemodynamically unstable.

4.3.3. Respiratory failure (who are in need for immediate ventilator support)

4.3.4. Shock not responding to initial fluid resuscitation.

4.3.5. Critically ill patients transferred from another hospital

4.3.6. Drug overdose (who are clinically unstable)

4.3.7. High Risk Pre-and post -operative patients

4.3.7.1. Surgical patients with cardiac disease

4.3.7.2. Patient with ejection fraction of less than 30%

4.3.8. **Medical cases such as:**

4.3.8.1. Malignant hypertension and hypertension emergencies

4.3.8.2. Acute renal failure

4.3.8.3. Acute severe asthma

4.3.8.4. Status epilepticus

4.3.8.5. Acid-base disturbance

4.3.8.6. Diabetic Ketoacidosis coma- who cannot protect his airway

4.4. **Admission Prioritization**

4.4.1. **First Priority**: Patient requiring intensive treatment. Comprises critically ill, unstable patient in need of and not limited to:

4.4.1.1. Ventilator support

4.4.1.2. Vasoactive drug infusion

4.4.1.2.1. Septic shock

4.4.1.2.2. Post-operative neurosurgical

4.4.1.2.3. Thoracic and major vascular surgery

4.4.2. **Second Priority:** Patient requires advanced monitoring who are at risk for the need of immediate intensive treatment

4.4.2.1. Patients with underlying heart, lung or renal disease who developed acute severe medical illness or undergone major surgery and not limited to.

4.4.3. **Third priority:** Critically unstable patients with severely reduced likelihood of recovery and / or benefit from ICU treatment.

4.4.3.1. Metastatic malignancy complicated by infection, cardiac tamponade or airway obstruction.

4.4.3.2. End-stage of heart or lung disease complicate by acute illness

4.5. **Types of patients that is not appropriate for ICU admission**

4.5.1. **Admission** of these patient categories should be on an individual basis, under unusual circumstances and the discretion of the ICU specialists/ Consultant.

4.5.1.1. Little or no anticipated benefit from ICU care based on low risk of active intervention, which includes patients with peripheral vascular surgery, hemodynamically stable diabetic ketoacidosis with mild to moderate metabolic acidosis, congestive heart failure, conscious drug overdose, etc.

4.5.1.2. Patient with terminal and irreversible illness facing imminent death.

4.5.1.2.1. Patient with decision making capacity who decline intensive care and-/ or invasive monitoring and who receive comfort care only, brain death, non - organ donors.

4.5.1.2.2. Patients in persistent vegetative state, patient who are permanently unconscious, etc., confirmed brain dead except for organ donation.

4.5.1.2.3. Refusing aggressive supportive therapy "Comfort care"

4.5.1.2.4. Vegetative state.

4.6. ICU admission by diagnostic category

4.6.1. **Factors to be considered when assessing suitability for admission to ICU:**

4.6.1.1. Diagnosis

4.6.1.2. Severity of illness

4.6.1.3. Age

4.6.1.4. Co-existing disease

4.6.1.5. Physiologic reserve

4.6.1.6. Prognosis

4.6.1.7. Availability of suitable treatment

4.6.1.8. Response to treatment to date

4.6.1.9. Recent cardiopulmonary arrest

4.6.1.10. Anticipated quality of life

4.6.2. **Diagnosis Model**

4.6.2.1. **Cardio-Vascular:**

4.6.2.1.1. Acute myocardial infarction

4.6.2.1.2. Unstable acute coronary syndrome

4.6.2.1.3. Acute pulmonary /cardiogenic edema

4.6.2.1.4. Life-threatening dysrhythmias

4.6.2.1.5. Pericarditis with effusion/ Hypertensive emergencies

4.6.2.1.6. Dissecting Aortic Aneurysms

4.6.2.1.7. Need for continuous invasive monitoring of cardiovascular system

4.6.2.1.8. Cardio-pulmonary Arrest

4.6.2.2. **Respiratory:**

4.6.2.2.1. Acute respiratory failure requiring ventilator support

4.6.2.2.2. Acute Pulmonary embolism with hemodynamic instability

4.6.2.2.3. Patients in an intermediate care unit who are demonstrating respiratory deterioration.

4.6.2.2.4. Massive hemoptysis

4.6.2.2.5. Respiratory failure with threatened airway

4.6.2.3. **Neurological Disorders:**

4.6.2.3.1. Deep coma: Metabolic, toxic, or anoxic who cannot protect his her airway

4.6.2.3.2. Intracranial hemorrhage with potential for herniation.

4.6.2.3.3. Acute subarachnoid hemorrhage.

4.6.2.3.4. Infections of nervous system (Meningitis, encephalitis) with altered

mental status and respiratory compromise

4.6.2.3.5. Central nervous system or neuromuscular disorder with deteriorating neurologic or pulmonary function

4.6.2.3.6. Status epilepticus

4.6.2.3.7. Brain dead or potentially brain dead patients who are being aggressively managed while determining organ donation status severe head injured patients.

4.6.2.4. **Drug ingestion and drug overdose:**

4.6.2.4.1. Hemodynamic ally unstable drug ingestion

4.6.2.4.2. Drug ingestion with significantly altered mental status with inadequate airway protection seizures following drug ingestion.

4.6.2.5. **Gastrointestinal:**

4.6.2.5.1. Life- threatening gastrointestinal bleeding including hypotension, angina, continued bleeding or with comorbid conditions

4.6.2.5.2. Fulminant hepatic failure with hemodynamic instability as a feature of metabolic encephalopathy

4.6.2.5.3. Severe acute pancreatitis with hemodynamic instability

4.6.2.5.4. Esophageal perforation with or without mediastinitis

4.6.2.6. **Endocrine:**

4.6.2.6.1. Diabetic Ketoacidosis complicated by hemodynamic instability, altered mental status, respiratory insufficiency or severe acidosis

4.6.2.6.2. Thyroid storm or myxedema coma with hemodynamic instability

4.6.2.6.3. Hyperosmolar state with coma and / or hemodynamic instability

4.6.2.6.4. Other endocrine problems such as adrenal crises with hemodynamic instability

4.6.2.6.5. Severe hyperkalemia with altered mental status, requiring hemodynamic monitoring

4.6.2.6.6. Hypo or Hypernatremia with seizures or altered mental status.

4.6.2.6.7. Hypo or Hypomagnesaemia with hemodynamic, respiratory compromise or dysrhythmias

4.6.2.6.8. Hypo or hyperkalemia with dysrhythmias or muscular weakness

4.6.2.6.9. Hypophosphatemia with muscular weakness

4.6.2.7. **Renal:**

4.6.2.7.1. Acute rhabdomyolysis with renal insufficiency

4.6.2.7.2. Oliguria (<0.5 ml/kg/hr)

4.6.2.7.3. Life threatening hyperkalemia (>6mmol/L) resistant to drug treatment

4.6.2.7.4. Rising plasma Urea or Creatinine, or both

4.6.2.7.5. Severe metabolic acidosis

4.6.2.7.6. Symptoms related to uremia (Pericarditis, Encephalopathy)

4.6.2.8. **Hematology:**

4.6.2.8.1. Severe coagulopathy and/or bleeding diathesis

4.6.2.8.2. Severe anemia resulting in hemodynamic and./or respiratory compromise

4.6.2.8.3. Severe complication of Sickle cell crisis

4.6.2.8.4. Hematological malignancies with multi-organ failure

4.6.2.9. **Surgical:**

4.6.2.9.1. Post-operative patient with hemodynamic instability requiring hemodynamic monitoring / ventilator support or extensive nursing care.

4.6.2.9.2. Patient with cardiac disease, arrhythmias decompensated heart failure or low ejection fraction

4.6.2.10. **Miscellaneous:**

4.6.2.10.1. Septic Shock with hemodynamic instability or with failing tissue perfusion

4.6.2.10.2. Hemodynamic monitoring requiring more than one inotropic support

4.6.2.10.3. Clinical condition requiring ICU level nursing care

4.6.2.10.4. Obstetrical complications such as (Pre)eclampsia, HELLP syndrome, hemorrhage, thrombi-embolic disorders, sepsis

4.6.2.10.5. Environment injuries (Lightning, near drowning hypo/ hyperthermia)

4.6.2.10.6. New experimental therapies with potential for complication.

4.6.3. **Objective Parameters Model:**

4.6.3.1. **Vital Signs:**

4.6.3.1.1. Pulse, 40 or >150 beats/ min

4.6.3.1.2. Systolic arterial pressure <80mmHg or 20mmHg below the patients usual pressure

4.6.3.1.3. Mean Arterial pressure <60mmHG

4.6.3.1.4. Diastolic Arterial Pressure > I 2OmmHG

4.6.3.1.5. Respiratory rate >35 breaths/ min signs of respiratory distress

4.6.3.2. **Laboratory Values (newly discovered)**

4.6.3.2.1. Serum Sodium<11Ommol/L or >179mmol/L

4.6.3.2.2. Serum Potassium <2.0mmol?L or >7.0 mmol/L

4.6.3.2.3. PaO2 <6.7kPa on FiO2>0.5

4.6.3.2.4. PH<7.1 or > 7.7

4.6.3.2.5. Serum Glucose >44mmol/L

4.6.3.2.6. Serum Calcium >30mmol/L

4.6.3.2.7. Toxic level of drug or other chemical substance in hemodynamic ally or neurologically compromised patients.

4.6.3.3. **Radiography/ Ultrasonography/ Tomography (newly discovered)**

4.6.3.3.1. Cerebral Vascular hemorrhage, contusion or subarachnoid hemorrhage with altered mental status or focal neurologic signs

4.6.3.3.2. Ruptured viscera, bladder liver, esophageal varices or uterus with hemodynamic instability.

4.6.3.3.3. Dissecting aortic aneurysm

4.6.3.4. **Physical Findings (Acute onset)**

4.6.3.4.1. Unequal pupils in an unconscious patient

4.6.3.4.2. Airway obstruction

4.6.3.4.3. Deep coma that is at risk of airway compromise

4.6.3.4.4. Prolonged and recurrent /continuous Seizures

4.6.3.4.5. Cyanosis uncorrected by 02 supplementation

4.6.3.4.6. Cardiac Tamponade

5. PROCEDURE

5.1. The patient's attending physician or his designee shall assess if the patient requires intensive care

5.2. Upon admission ICU Nurse must assess and collect the patient's most abnormal values of physiological value, age and chronic health status (APACHE II tool) during their first 24 hours in the Intensive Care Unit. Add the point from the Acute Physiology Score (0-71), Age points (0-6), and Chronic Health points (0-5). Calculate and Interpretation of total score, higher scores mean higher predicted risk of hospital death.

5.3. Total APACHE II Score = APS + Age Point + Chronic Health Points.

5.4. Scoring range: < 10 point = < 15% , 20-24 = 40%, 30-34= 75%, >34 =>85%.

5.4.1. ICU Nurse must fill up of following information of the patient on the form such :

5.4.1.1. Patient name

5.4.1.2. Primary diagnosis

5.4.1.3. MRN

5.4.1.4. Age

5.4.1.5. ICU Admission date

5.4.1.6. Sex

5.4.1.7. Date scored

5.4.1.8. Scored by

5.4.2. A. Acute Physiology Score (APS)

5.4.3. APS Subtotal + GCS Score = APS Resulted

5.4.3.1. Temperature (Celsius)

5.4.3.1.1. Temp ≥ 41 = 4

5.4.3.1.2. Temp 39 – 40.9 = 3

5.4.3.1.3. Temp 38.5 – 38.9 = 1

5.4.3.1.4. Temp 36 – 38.4 = 0

5.4.3.1.5. Temp 34 – 35.9 = 1

5.4.3.1.6. Temp 32 – 33.9 = 2

5.4.3.1.7. Temp 30 – 31.9 = 3

5.4.3.1.8. Temp <29.9 = 4

5.4.3.2. Mean arterial pressure (mmHg)

5.4.3.2.1. MAP ≥ 160 = 4

5.4.3.2.2. MAP 130 – 159 = 3

5.4.3.2.3. MAP 110 – 129 = 2

5.4.3.2.4. MAP 70 – 109 = 0

5.4.3.2.5. MAP 50 – 69 = 2

5.4.3.2.6. MAP ≤ 49 = 4

5.4.3.3. Heart rate (beats/min)

5.4.3.3.1. HR ≥ 180 = 4

5.4.3.3.2. HR 140 179 = 3

5.4.3.3.3. HR 110 – 139 = 2

5.4.3.3.4. HR 70 – 109 = 0

5.4.3.3.5. HR 55 – 69 = 2

5.4.3.3.6. HR 40 – 54 = 3

5.4.3.3.7. HR ≤ 39 = 4

5.4.3.4. Respiratory Rate (beats /min)

5.4.3.4.1. RR ≥ 50 = 4

5.4.3.4.2. RR 35 – 49 = 3

5.4.3.4.3. RR 25 – 34 = 1

5.4.3.4.4. RR 12 – 24 = 0

5.4.3.4.5. RR 10 -11 = 1

5.4.3.4.6. RR 6 – 9 =2

5.4.3.4.7. RR ≤ 5 = 4

5.4.3.5. Oxygenation A-aDO2 if FiO2 ≥ 0.50 (mmHg)

5.4.3.5.1. A-aDO2 ≥ 500 = 4

5.4.3.5.2. A-aDO2 350 – 499 = 3

5.4.3.5.3. A-aDO2 200 – 349 = 2

5.4.3.5.4. A-aDO2 <200 = 0

5.4.3.6. Oxygenation PaO2 if FiO2 <0.50 (mmHg)

5.4.3.6.1. PaO2 ≥ 70 = 0

5.4.3.6.2. PaO2 61 – 70 = 1

5.4.3.6.3. PaO2 55 – 60 = 2

5.4.3.6.4. PaO2 <55 = 4

5.4.3.7. Arterial PH

5.4.3.7.1. pH ≥ 7.70 = 4

5.4.3.7.2. pH 7.60 – 7.69 = 3

5.4.3.7.3. pH 7.50 – 7.59 = 1

5.4.3.7.4. pH 7.33 – 7.49 = 0

5.4.3.7.5. pH 7.25 – 7.32 = 2

5.4.3.7.6. pH 7.15 – 7.24 = 3

5.4.3.7.7. pH < 7.15 = 4

5.4.3.8. Serum HCO3 (mmol/L)

5.4.3.8.1. HCO3 ≥ 52 = 4

5.4.3.8.2. HCO3 41 – 51.9 = 3

5.4.3.8.3. HCO3 32 – 40.9 = 1

5.4.3.8.4. HCO3 22 – 31.9 = 0

5.4.3.8.5. HCO3 18 – 21.9 = 2

5.4.3.8.6. HCO3 15 – 17.9 =3

5.4.3.8.7. HCO3 <15 = 4

5.4.3.9. Serum Sodium (mmol/L)

5.4.3.9.1. Na+ ≥ 180 = 4

5.4.3.9.2. Na+ 160 – 179 = 3

5.4.3.9.3. Na+ 155 – 159 = 2

5.4.3.9.4. Na+ 150 – 154 =1

5.4.3.9.5. Na+ 130 – 149 = 0

5.4.3.9.6. Na+ 120 – 129 = 1

5.4.3.9.7. Na+ 111 – 119 = 3

5.4.3.9.8. Na+ ≤ 110 = 4

5.4.3.10. Serum Potassium (mmol/L)

5.4.3.10.1. K+ ≥ 7 = 4

5.4.3.10.2. K+ 6 – 6.9 = 3

5.4.3.10.3. K+ 5.5 – 5.9 = 1

5.4.3.10.4. K+ 3.5 – 5.4 = 0

5.4.3.10.5. K+ 3 – 3.4 = 1

5.4.3.10.6. K+ 2.5 – 2.9 = 2

5.4.3.10.7. K+ <2.5 = 4

5.4.3.11. Serum Creatinine (mg/dL)

5.4.3.11.1. Crea ≥ 3.5 = 4

5.4.3.11.2. Crea 2.0 – 3.4 = 3

5.4.3.11.3. Crea 1.5 – 1.9 = 2

5.4.3.11.4. Crea 0.6 – 1.4 = 0

5.4.3.11.5. Crea <0.6 = 2

5.4.3.12. Hematocrit (%)

5.4.3.12.1. HCT ≥ 60 = 4

5.4.3.12.2. HCT 50 – 59.9 = 2

5.4.3.12.3. HCT 46 – 49.9 = 1

5.4.3.12.4. HCT 30 – 45.9 = 0

5.4.3.12.5. HCT 20 – 29.9 = 2

5.4.3.12.6. HCT < 20 = 4

5.4.3.13. WBC Count

5.4.3.13.1. WBC ≥ 40 = 4

5.4.3.13.2. WBC 15 – 39.9 = 2

5.4.3.13.3. WBC 3 – 14.9 = 0

5.4.3.13.4. WBC 1 – 2.9 = 2

5.4.3.13.5. WBC <1 = 4

5.4.3.14. Neurological Score Glasgow Coma Scale (GCS)

5.4.3.14.1. 15 = 0

5.4.3.14.2. 14 = 1

5.4.3.14.3. 13 = 2

5.4.3.14.4. 12 = 3

5.4.3.14.5. 11 = 4

5.4.3.14.6. 10 = 5

5.4.3.14.7. 9 = 6

5.4.3.14.8. 8 = 7

5.4.3.14.9. 7 = 8

5.4.3.14.10. 6 = 9

5.4.3.14.11. 5 = 10

5.4.3.14.12. 4 = 11

5.4.3.14.13. 3 = 12

5.4.4. B. Age Point

5.4.4.1. . ≤ 40 = 0

5.4.4.2. . 45 – 54 = 2

5.4.4.3. . 55 – 64 = 3

5.4.4.4. . 65 – 74 = 5

5.4.4.5. . ≥ 75 = 6

5.4.5. C. Chronic Health Points

5.4.5.1. Severe organ insufficiency of Immunocompromised

5.4.5.1.1. Non-operative/ emergency post OP = 5

5.4.5.1.2. Elective postoperative = 2

5.4.5.1.3. None = 0

5.5. The attending physician or his designee shall request ICU physician evaluation for admission when indicated by filling up a consultation form indicating the reason for evaluation and/or notifying the ICU physician thru phone about the request and patient's condition.

5.6. Upon request for patient admission, ICU physician will evaluate the case in the requesting unit/ward.

5.7. Upon the discretion/evaluation of the ICU physician, if the patient does not meet the ICU admission criteria, but need prompt intervention, he will manage the case in the requesting unit/ward and re-evaluate after

5.8. Upon the discretion/evaluation of the ICU physician and the patient conforms to ICU admission criteria, he will stabilize the patient in the unit according to priority and inform the attending

physician or his designee to shift the patient to ICU.

5.9. The attending physician or his designee will obtain consent for CRITICAL CARE admission that explains about Potentially Life-Sustaining treatment in ICU. "potential" is used to emphasize the fact that there exists no certainty that any treatment will be successful, and that success rates vary widely between and within treatment types. Obtaining of Critical Care Consent Form is witness by the ward/unit Nurse on Duty

5.10. The ICU physician will then inform the ICU Charge Nurse regarding the incoming admission.

5.11. The ward/unit Nurse on Duty will inform the ICU Charge Nurse regarding admission and patient's condition and needed equipment.

5.12. ICU charge nurse will prepare the bed for the new admission, respiratory therapist or trained ICU staff will prepare ventilator if needed.

5.13. When demand for beds exceeds availability, the Chief of ICU or his designee determines which patient is to be given priority

5.14. Patient will be transported to ICU accompanied by the assigned NOD

5.15. ICU charge nurse and assigned nurse will assist unit/ward staff with the transfer; both assess patient thoroughly, attach necessary medical equipment as needed; assigned nurse checks Critical Care Consent Form is secured upon receiving patient.

5.16. Transfer form will be filled up by the assigned NOD in the ward.

5.17. Proper documentation and thorough endorsement should be observed as part of assessment for patient who is in critical condition.

5.18. Transfer patient's account in the Medical Clod Care System.

5.19. Bed management logbook will be submitted to the nursing supervisor.

5.20. The Assistant Executive Director for Medical Services might set a committee to retrospectively review cases wherein the attending physician has disagreed with the decision of the ICU specialist/ consultant.

6. MEASUREMENT

6.1. 100% of the medical and nursing staff are well oriented and adheres to this policy.

6.2. 100% of ICU patients are evaluated using the APACHE II Score Calculator.

7. REFERENCES /CROSS- REFERENCES

7.1. PMSH-APPH-QPS-0011(v6) Patient Admission, 21/12/2025, p.1-14

7.2. PMSH-DPP-ICU-0001(v6) Admission Criteria, 09/02/2025, p.1-9

7.3. https://deposit.com/calc/apache-ii-score-calculator/

8. ATTACHMENT

8.1. Critical Care Consent Form

8.2. Attachment 001 – DPP-ICU-001-APACHE II Score

9. REVISION CLAUSE

9.1. Modifications of definition & abbreviation, responsibility, policy and procedure adding APACHE contents